INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Weekly cetuximab

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
7 days
See the active treatment phase

Treatment stages
1
Explained in the treatment reference

Regimen source
NCCP 00207
Version 6

> One larger first infusion, then cetuximab once every 7 days while treatment is helping and side effects remain acceptable. This is cetuximab alone, with no home chemotherapy pump.

Treatment stages

01 **A 7-day reference cycle**
Clinic → home → review

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name _____

Hospital / oncology team _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

EGFR-expressing RAS wild-type metastatic colorectal cancer after the specified previous therapies

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. New breathing difficulty, chest pain, collapse, severe diarrhoea or inability to drink require prompt assessment. Report eye pain, blurred vision, blistering or infected skin promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

How your medicines work

To control EGFR-expressing RAS wild-type bowel cancer that has spread after oxaliplatin- and irinotecan-based treatments have failed and irinotecan cannot be tolerated.

EGFR-TARGETED ANTIBODY

Cetuximab

Erbitux



Blocks growth messages at the cell surface

Cetuximab attaches to EGFR on the outside of cells and blocks growth messages. In bowel cancer, a validated RAS wild-type result helps identify whether this approach is suitable. Healthy skin also uses EGFR, so skin and nail effects need early care.

EGFR receives a growth signal → Cetuximab blocks the receptor → Some growth signals are reduced

What to watch for

Report rash or sore nails early. Tell the nurse immediately about infusion symptoms. Eye pain, new breathing problems or severe diarrhoea need prompt advice. Blood tests check minerals during treatment and for two months afterwards.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Your treatment aim

The setting changes what benefit means.

Why this option?

Cetuximab alone is an option for RAS wild-type bowel cancer that has spread after oxaliplatin and irinotecan have failed, when irinotecan cannot be tolerated. Your team explains the aim, alternatives and how benefit will be reviewed.

A plan that can change

For bowel cancer, discuss control of cancer and symptoms, and whether further local treatment may become possible. Your preferences and priorities matter.

Why this treatment is offered

In selected previously treated bowel cancer, cetuximab improved disease control and survival compared with supportive care.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

Treatment, recovery and review

Weekly cetuximab

7-day view



This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

CLINIC → HOME → REVIEW

A 7-day reference cycle

D1 D7

Cetuximab on Day 1. No home chemotherapy pump. A repeating 7-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

This is a general explanation, not an appointment calendar. Delays or changes are decided by your team. Each medicine can need a different change.

BASELINE → TREATMENT → FOLLOW-UP

Treatment checks across the course

Relative stages, not equally spaced dates. Agree who books each check, reviews results and gives advice if a check is missed.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00207 v6 · official regimen

Tests in your plan

Carcinoembryonic antigen (CEA) — colorectal plans

If part of your plan

WHEN

For a colorectal cancer plan, if your team uses CEA; your team sets the interval.

A marker trend interpreted alongside symptoms, examination and scans.

Record the baseline, latest result and next date. A marker change alone does not establish progression.

THE WHOLE TREATMENT JOURNEY

Weekly cetuximab: treatment and checks

Selected metastatic colorectal cancer; or locally advanced head/neck cancer with radiotherapy when platinum is unsuitable

One larger first infusion is followed by weekly infusions. Bowel-cancer treatment continues while helping and tolerated. For head-and-neck treatment, cetuximab starts one week before radiotherapy and continues until radiation treatment finishes.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	First treatment	During treatment	Phase / dose review	After treatment
Bloods and symptoms NCCP 00207	Baseline blood count, kidney and liver checks	Confirm tests and prescription before the first dose	Repeat bloods and symptom checks according to this regimen	Recheck after a hold or treatment change	Follow unresolved effects and the agreed disease-review plan
Treatment review NCCP 00207	Confirm exact cancer setting and biomarkers	Explain expected benefit and how it will be assessed	Report new symptoms between visits	Review response, tolerability and active medicines	Agree the next treatment or surveillance plan
Electrolytes and EGFR effects NCCP 00207	Check electrolyte and skin-risk plan	Skin care and prompt reaction reporting	Monitor magnesium, calcium, potassium, skin and eyes	Do not stop follow-up because the infusion stops	Monthly electrolytes, magnesium and calcium for two months after the last EGFR-antibody dose

What each check is for

1 **A 7-day reference cycle**

Clinic → home → review

For selected bowel cancer that has spread, cetuximab aims to control the cancer after previous treatments. For selected locally advanced head-and-neck cancer when platinum chemotherapy is unsuitable, it is combined with radiotherapy. Your team confirms which pathway applies.

Ask / act: Confirm the current phase, prescription, tests and support medicines.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Infusion visits and care between treatments

Cetuximab on Day 1. No home chemotherapy pump. A repeating 7-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

STEP 2

Checks after treatment

Monthly electrolytes, magnesium and calcium for two months after the last cetuximab dose; assign responsibility before treatment ends.

STEP 3

Access and practical support

Day-unit intravenous (IV) access; no home fluorouracil pump.

Do I need a port?

A peripheral cannula or existing central line may be used. A port is not automatically needed; the team considers your veins and overall treatment plan.

If the schedule changes

Ask the unit to confirm the next visit and which medicine is being changed. Do not copy dates from another regimen.

Observation at every antibody visit

Premedicines and monitoring add time to every cetuximab visit. Your team will confirm the observation period before you leave; do not assume that later visits can omit observation.

Before the first treatment

Checks reduce avoidable risk.

Blood tests and medicines

Review blood counts, kidney/liver tests, symptoms, allergies and all medicines and supplements. Tell the team about red-meat allergy or tick bites before cetuximab because these can be associated with infusion-reaction risk.

Preparing access

Allow for blood tests, review, pharmacy preparation, infusion and observation. A port is not automatically required; ask which access is suitable.

Visits and treatment sequence

Know which steps happen in the clinic and at home.

When treatment changes or ends

Follow-up continues.

Symptoms and monitoring

Continue the cetuximab skin plan and mineral checks for two months after the last dose. Ask which medicines to continue, when to seek help and how response will be assessed.

Next decisions

The team reviews benefit, side effects and your priorities. Supportive and palliative care can help alongside active treatment. Ask for a clear next-step and contact plan.

CHAPTER 03

Symptoms and supportive care

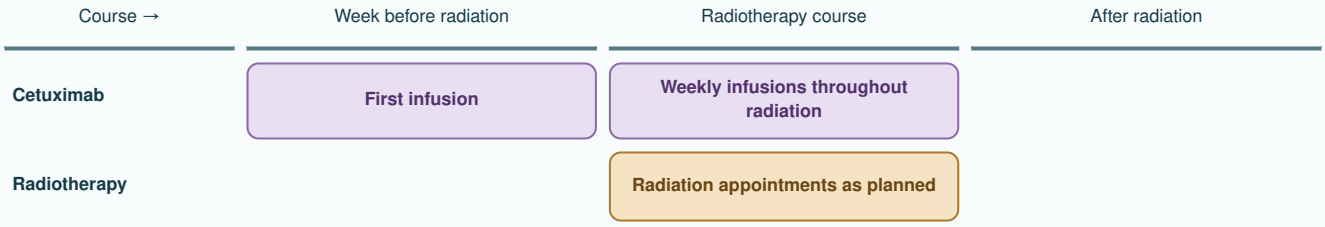
IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

THE WHOLE TREATMENT COURSE

Head-and-neck indication: cetuximab before and during radiotherapy

This panel applies only to the head-and-neck radiotherapy indication of NCCP 00207. Cetuximab is a targeted antibody. Colorectal cetuximab alone has no routine radiotherapy component.



The axis shows order, not equal durations. Cetuximab starts one week before radiation and continues until radiation ends. The radiation team sets the number of sessions and confirms eligibility.

> **After the course: continue skin, mouth, swallowing and nutrition support. Mineral monitoring continues for two months after the final cetuximab dose.**

Educational reference, not a personal appointment calendar. NCCP 00207 v5, page 1; current EMA Erbitux information

Day 1 → Day 7: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

One larger first infusion, then cetuximab once every 7 days while treatment is helping and side effects remain acceptable. This is cetuximab alone, with no home chemotherapy pump.

Treatment or check	Timing in this view
Cetuximab infusion	 **Day 1** intravenous (IV)
Skin, nails and minerals	 **Days 1–7** Can build over successive cycles
Energy and bowel symptoms	 **Days 1–7** Variable timing

Your planned visits: Cetuximab on Day 1. No home chemotherapy pump. A repeating 7-day cycle. The number of cycles depends on benefit, side effects and your treatment plan.

Care continues: Continue the skin-care plan and mineral checks arranged after cetuximab. Ask which medicines to continue and when your next review is planned.

No guaranteed safe days: An infection, reaction or other urgent problem can occur at any time. The graph does not predict a personal low point.

Why these bands stay broad: NCCP defines the treatment days. Symptom bands are broad educational guidance; reactions can occur on any day, and cumulative effects may continue between cycles. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00207 v6 · official regimen ↗

There are no guaranteed "safe days". Fever, shivers or suddenly feeling unwell require immediate contact. Serious symptoms override any timeline or diary.

Between-treatment notes

Day 1: Treatment and checks

The team reviews blood tests, symptoms and your prescription before treatment. Premedicines and observation are part of cetuximab visits. Know the contact number and the next planned visit.

Day 2–3: Care between visits

Follow the skin-care or bowel-symptom plan provided. There is no pump to remove. Call early for symptoms; do not wait for the next appointment.

Day 4–7: Recovery and the next review

The team uses symptoms, tests and your priorities to decide what happens next. Bring questions and a symptom record if useful; this is optional.

Support around the medicines

1 First cetuximab dose

Skin care and sun protection under local EGFR policy. Premedicines and monitoring add time to every cetuximab visit. Your team will confirm the observation period before you leave; do not assume that later visits can omit observation. Report delayed reaction symptoms urgently. Monthly electrolytes/Mg/Ca for two months after last dose.

2 Subsequent weekly cetuximab

Premedication continues for subsequent infusions. Post-treatment monthly electrolytes/Mg/Ca for two months.

3 Previous infusion reactions

Tell the team about any previous severe antibody reaction, even if you feel well now. Premedication does not remove all reaction risk. Report breathing problems, swelling, faintness or rapidly worsening symptoms immediately.

Symptoms in context

Report what changed and how it affects you.

Bowel symptoms

Follow your written rescue plan. Fever, blood, severe pain, reduced urine or inability to drink needs urgent advice.

Skin and nail changes

Start the prescribed skin-care plan early, use moisturiser and sun protection, and report painful or infected skin or nails. Keep mineral blood-test appointments during treatment and for two months after cetuximab.

Living with treatment

Use the support available.

Food, activity and practical help

Use prescribed anti-sickness medicines, pace activities and ask for nutritional, emotional or practical support.

Pregnancy and fertility

Discuss fertility preservation and reproductive plans before treatment. Ask for the contraception and breastfeeding instructions for every medicine; the whole regimen may require longer precautions than one medicine alone.

Questions worth asking

Do I need to enter dates to understand the cycle?

No. The graph explains a general pattern. Your hospital arranges appointments; the guide and its safety information are available without entering anything.

Will I have a pump at home?

There is no home fluorouracil pump in this regimen. Your team chooses a cannula or central line based on your veins and wider treatment plan.

What happens if a dose is delayed?

The team checks the cause and may delay, reduce, change or stop one medicine. Ask which component changes and when you should next attend; do not restart yourself.

How will I know whether it is helping?

Scans, symptoms and clinical review assess benefit. A rash, side effect or good day alone cannot show whether the cancer is responding.

The graphic below shows the relationship.

Why is RAS testing mentioned?

RAS testing selects cetuximab treatment in colorectal cancer. Confirm the cancer-specific plan with your team.

> **Try explaining the plan back:** Can you explain which visits are planned, whether a pump is involved, and which symptoms mean you should call today? No dates or symptom scores are needed to use this guide.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

First cetuximab dose

Active medicines: cetuximab

Emetogenic risk: low

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV)	60 minutes before cetuximab NCCP suggested premedication.
Dexamethasone	8 mg IV	60 minutes before cetuximab NCCP suggested mandatory infusion premedication; reconcile low-risk antiemetic support to avoid duplicate steroid.
Rescue antiemetic	As individually prescribed	If required Low emetic risk; use current NCCP/local guidance and account for the infusion steroid.

Additional phase-specific support

- Skin care and sun protection under local EGFR policy.
- First two doses require post-infusion observation; delayed-reaction education.
- Monthly electrolytes/Mg/Ca for two months after last dose.
- NCCP p1 permits cessation of observation after two tolerated cetuximab doses, whereas Erbitux SmPC section 4.2 requires monitoring during and for at least one hour after each infusion. Reconcile and document the local observation plan; do not promise early discharge.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Subsequent weekly cetuximab

Active medicines: cetuximab

Emetogenic risk: low

Medicine	Dose / route	When
Chlorphenamine	10 mg intravenous (IV)	60 minutes before each infusion
Dexamethasone	8 mg IV	60 minutes before each infusion Reconcile antiemetic steroid; no automatic duplication.

Additional phase-specific support

- Premedication continues for subsequent infusions.
- Post-treatment monthly electrolytes/Mg/Ca for two months.
- NCCP p1 permits cessation of observation after two tolerated cetuximab doses, whereas Erbitux SmPC section 4.2 requires monitoring during and for at least one hour after each infusion. Reconcile and document the local observation plan; do not promise early discharge.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Eyes

Cetuximab

Eye pain or new visual change needs prompt advice.

2 Blood minerals

Cetuximab

Tests check magnesium and other minerals even when you feel well.

3 Breathing / reaction

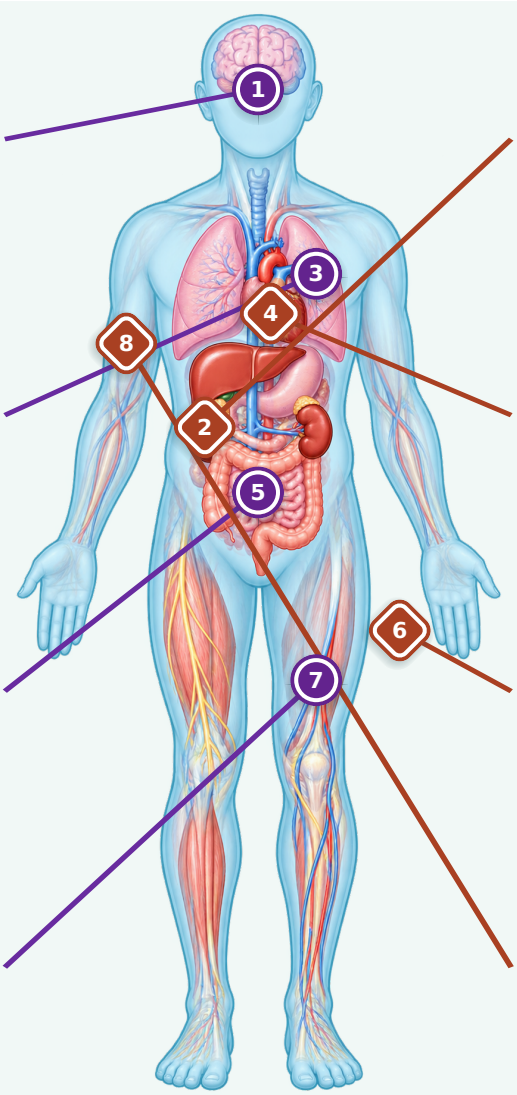
Cetuximab

New breathing difficulty requires prompt assessment.

4 Infusion symptoms

Cetuximab

Tell the nurse immediately about itching, swelling or flushing.



5 Bowel

Treatment

Diarrhoea, increased stoma output or dehydration: call early.

6 Skin / nails

Cetuximab

Report painful rash, cracks or infected skin early.

7 Blood / infection

Treatment

Fever, shivers or sudden illness: contact oncology immediately.

8 Infusion access

Treatment delivery

Pain, swelling, leakage or a loose connection needs immediate advice.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

New breathing difficulty, chest pain, collapse, severe diarrhoea or inability to drink require prompt assessment. Report eye pain, blurred vision, blistering or infected skin promptly.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **NCCP 00207 v5, page 1; current EMA Erbitux information**
   https://assets.hse.ie/media/documents/207_v5_Cetuximab_7day.pdf
2. **National Cancer Control Programme (NCCP) 00207 v6 · official regimen**
   https://healthservice.hse.ie/documents/6941/207_Cetuximab_7day_.pdf
3. **HSE: chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
4. **EMA: Erbitux product information and patient leaflet**
   https://www.ema.europa.eu/en/documents/product-information/erbitux-epar-product-information_en.pdf
5. **CO.17 KRAS analysis — primary report**
   https://pubmed.ncbi.nlm.nih.gov/18946061/
6. **NCCP emetogenic classification V6**
   https://healthservice.hse.ie/documents/7152/Classification_Document_for_Systemic_Anti-_Cancer_Therapy_systemic anticancer therapy (SACT)_Induced_Nausea__pWgDCrm.pdf
7. **NCCP NCIS Medical Oncology antiemetics V8**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
8. **NCI: colon cancer treatment — carcinoembryonic antigen (CEA) and follow-up**
   https://www.cancer.gov/types/colorectal/hp/colon-treatment-pdq